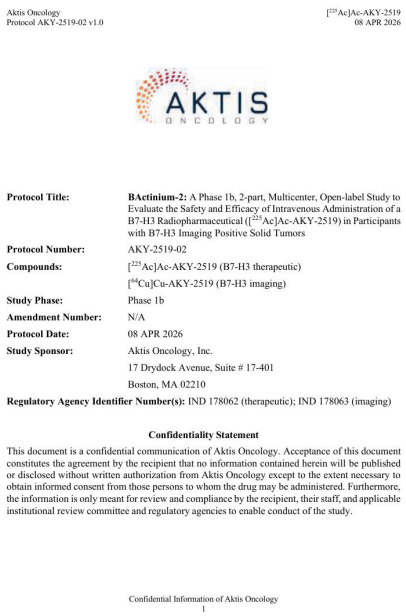
BActinium-2

AKY-2519-02 · 64Cu / 225Ac AKY-2519 B7-H3 · Phase 1b

Aktis Oncology · P02 in Board Studio · 08 APR 2026 · 118 pages



Aktis Oncology
Protocol AKY-2519-02 v1.0

[225Ac]Ac-AKY-2519
08 APR 2026

AKTIS ONCOLOGY

Protocol Title: **BActinium-2:** A Phase 1b, 2-part, Multicenter, Open-label Study to Evaluate the Safety and Efficacy of Intravenous Administration of a B7-H3 Radiopharmaceutical ([225Ac]Ac-AKY-2519) in Participants with B7-H3 Imaging Positive Solid Tumors

Protocol Number: AKY-2519-02

Compounds: [225Ac]Ac-AKY-2519 (B7-H3 therapeutic)
[64Cu]Cu-AKY-2519 (B7-H3 imaging)

Study Phase: Phase 1b

Amendment Number: N/A

Protocol Date: 08 APR 2026

Study Sponsor: Aktis Oncology, Inc.
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Regulatory Agency Identifier Number(s): IND 178062 (therapeutic); IND 178063 (imaging)

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Confidential Information of Aktis Oncology
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What this study is, in one breath

Copper-64 B7-H3 PET to pick imaging-positive solid tumors, then actinium-225 on the same 49-amino-acid miniprotein every 4 weeks.

Alpha miniprotein therapy plus companion PET. Not an antibody. Not PSMA. Not lutetium.

> **If the protocol feels like a wall of words**
>
> A protocol is a recipe: who can join, what is injected, how much, when you scan or treat, and how you decide if a picture or outcome was right. This sheet is the same idea in normal English. It does not replace the signed site copy.

1. Two products, one 49-aa miniprotein

Aktis BActinium-2 / AKY-2519-02 is Phase 1b of a B7-H3 (CD276) radiopharmaceutical. Same synthetic 49-amino-acid miniprotein with a DOTA-PEG4 spacer, two labels. Not an antibody, not PSMA, not DLL3, not tarlatamab.

IMP	Role	Starting numbers
[64Cu]Cu-AKY-2519	Companion PET. T1/2 12.7 h.	148-222 MBq (4-6 mCi) IV. Gate: uptake > background in >=1 lesion at 90-180 min.
[225Ac]Ac-AKY-2519	Therapy. Alpha. T1/2 9.9 d.	Start 6 MBq IV bolus Q4W. BOIN 6 / 9 / 12 MBq (DL-1 = 4; intermediates 7.5 / 10.5).

Store both upright in lead at 15-30 C. Ac-225 use-by 96 h from end of production; Cu-64 48 h. IND 178062 (therapy) / 178063 (imaging).

2. Who

Adults with B7-H3 PET-positive locally advanced or metastatic NSCLC, ES-SCLC, mCRPC, HNSCC, ESCC, or CRC after therapies known to help that tumor. ECOG 0-1. ~20-25 sites. Up to 152 enrolled (~119 unique after rollover).

- NSCLC: progressed on PD-(L)1 and chemotherapy (together or separate).
- eGFR >= 60. Hb >= 9, ANC >= 1.5, platelets >= 100, no transfusion or G-CSF in 7 days.
- Hard exclusions: any prior radioligand therapy, any prior B7-H3 targeted therapy, pregnancy, QTcF >470 ms.

3. Design

Open-label two-part Phase 1b. Part 1: BOIN dose escalation (n ~21, 3 evaluable per level, <=1 mCRPC or CRC per dose) plus backfill at two SRC-chosen levels (up to 30 each, >=15 NSCLC, <=3 mCRPC). Part 2 at the recommended expansion dose: Cohort 1 up to 31 NSCLC; Cohort 2 up to 40 other PET-positive solid tumors, no mCRPC, <=20 of any one type. BOP2 futility.

- Up to 4 cycles >=4 weeks apart. Two extra cycles only if dose <=9 MBq and clinical benefit. No intra-patient dose escalation.
- Disease imaging every 8 +/- 1 weeks (CT; bone scan if mCRPC). RECIST 1.1 or PCWG3.
- ~10 patients at select sites: Cu-64 PET x3 and Ac-225 SPECT x3 (Table 20). Everyone else only needs the 90-180 min PET gate.

4. DLT window is 4 weeks after first Ac-225

Bucket	Counts as DLT
Heme	G4 ANC or platelets >7 days; >=G3 TCP with clinically significant bleed; febrile neutropenia any grade.
Non-heme	>=G3 with listed exceptions (short nausea/diarrhea, <1 week G3 fatigue, brief electrolytes, asymptomatic lipase). Hy's law.
Other	>14-day delay or permanent stop; death not clearly PD.

Miniprotein alpha, not lutetium and not an antibody

Cu-64 PET is the gate. Ac-225 is the treatment. Do not copy BET-121 GBq numbers, VECTRA 12 MBq PSMA, or P1239 0.75 uCi/kg antibody doses.

What to know cold

Q. Target?
A. B7-H3 / CD276 on a 49-aa DOTA-PEG4 miniprotein. Not an antibody.

Q. Imaging?
A. [64Cu]Cu-AKY-2519, 4-6 mCi. Gate: uptake > background at 90-180 min.

Q. Therapy?
A. [225Ac]Ac-AKY-2519 start 6 MBq IV Q4W. BOIN 6 / 9 / 12 MBq.

Q. Who / n?
A. PET+ NSCLC, ES-SCLC, mCRPC, HNSCC, ESCC, CRC. ~152 enrolled.

Q. DLT?
A. 4-week window after first Ac-225. G4 ANC/TCP >7 d; febrile neutropenia.

How to use this with Board Studio

Open the matching protocol module and press play. The left pane is the signed page. Use this PDF when legalese stalls you. If anything conflicts, the signed protocol wins.

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